

L23 - FPSC DELHI ROAD
46,MANBAGH CHORAHA,
JAISINGHPURA KHOR
JAIPUR

Name	: Baby MANSWI JAIN	Collected	: 28/8/2021 1:55:00PM
Lab No.	: 314643947	Received	: 28/8/2021 2:18:26PM
Age: 8 Years	Gender: Female	Reported	: 29/8/2021 2:03:36PM
A/c Status : P	Ref By : Dr. SELF	Report Status	: Final

Test Name	Results	Units	Bio. Ref. Interval
SwasthFit Super 4			

COMPLETE BLOOD COUNT;CBC
(Electrical Impedence,Manual)

Hemoglobin	13.60	g/dL	11.50 - 15.50
Packed Cell Volume (PCV)	41.00	%	35.00 - 45.00
RBC Count	4.89	mill/mm3	4.00 - 5.20
MCV	84.00	fL	77.00 - 95.00
MCH	27.70	pg	25.00 - 33.00
MCHC	33.10	g/dL	31.00 - 37.00
Red Cell Distribution Width (RDW)	11.10	%	11.90 - 14.90
Total Leukocyte Count (TLC)	10.10	thou/mm3	5.00 - 13.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	50.90	%	32.00 - 71.00
Lymphocytes	42.60	%	20.00 - 59.00
Monocytes	2.70	%	2.00 - 10.00
Eosinophils	2.90	%	0.00 - 8.00
Basophils	0.90	%	0.00 - 1.00
Absolute Leucocyte Count			
Neutrophils	5.14	thou/mm3	2.00 - 8.00
Lymphocytes	4.30	thou/mm3	1.00 - 5.00
Monocytes	0.27	thou/mm3	0.20 - 1.00
Eosinophils	0.29	thou/mm3	0.10 - 1.00
Basophils	0.09	thou/mm3	0.02 - 0.10
Platelet Count	350.0	thou/mm3	170.00 - 450.00
Mean Platelet Volume	8.3	fL	6.5 - 12.0

Note



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Test Name	Results	Units	Bio. Ref. Interval
1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood			
2. Test conducted on EDTA whole blood			



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Test Name	Results	Units	Bio. Ref. Interval
LIVER & KIDNEY PANEL, SERUM			
Bilirubin Total* (DPD)	0.43	mg/dL	0.30 - 1.20
Bilirubin Direct* (DPD)	0.10	mg/dL	<0.20
Bilirubin Indirect* (Calculated)	0.33	mg/dL	<1.10
AST (SGOT)* (IFCC without P5P)	137	U/L	<35
ALT (SGPT)* (IFCC without P5P)	168	U/L	<35
GGTP* (IFCC)	29	U/L	4 - 22
Alkaline Phosphatase (ALP)* (IFCC-AMP)	288	U/L	69 - 325
Total Protein* (Biuret)	7.21	g/dL	6.00 - 8.00
Albumin* (BCG)	4.14	g/dL	3.80 - 5.40
A : G Ratio* (Calculated)	1.35		0.90 - 2.00
Urea* (Urease UV)	20.55	mg/dL	10.00 - 38.00
Creatinine* (Modified Jaffe,Kinetic)	0.43	mg/dL	0.30 - 0.70
Uric Acid* (Uricase)	5.01	mg/dL	2.60 - 6.00



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Test Name	Results	Units	Bio. Ref. Interval
Calcium, Total* (Arsenazo III)	9.25	mg/dL	8.80 - 10.80
Phosphorus* (Molybdate UV)	4.27	mg/dL	3.20 - 5.80
Sodium* (Indirect ISE)	140.00	mEq/L	138.00 - 145.00
Potassium* (Indirect ISE)	4.34	mEq/L	3.40 - 4.70
Chloride* (Indirect ISE)	104.00	mEq/L	101.00 - 109.00



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Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC)			
HbA1c	5.8	%	4.00 - 5.60
Estimated average glucose (eAG)	120	mg/dL	

Interpretation
HbA1c result is suggestive of at risk for Diabetes (Prediabetes)/ well controlled Diabetes in a known Diabetic

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient’s blood glucose levels.

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants,elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g.,recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used.Iron deficiency anemia is associated with higher HbA1c



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Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE, FASTING (F), PLASMA* (Hexokinase)	102.00	mg/dL	70.00 - 100.00
VITAMIN B12; CYANOCOBALAMIN, SERUM* (CLIA)	357.00	pg/mL	211.00 - 911.00

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity

VITAMIN D, 25 - HYDROXY, SERUM* (Chemiluminescence)	86.40	nmol/L	75.00 - 250.00
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Interpretation

LEVEL	REFERENCE RANGE IN nmol/L	COMMENTS
Deficient	< 50	High risk for developing bone disease
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient	75-250	Optimal concentration for maximal health benefit
Potential intoxication	>250	High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.



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Test Name	Results	Units	Bio. Ref. Interval
	- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L. - It shows seasonal variation, with values being 40-50% lower in winter than in summer. - Levels vary with age and are increased in pregnancy. - A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available		

Comments

Vitamin D promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

Decreased Levels

- Inadequate exposure to sunlight
- Dietary deficiency
- Vitamin D malabsorption
- Severe Hepatocellular disease
- Drugs like Anticonvulsants
- Nephrotic syndrome

Increased levels

Vitamin D intoxication



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Test Name	Results	Units	Bio. Ref. Interval
THYROID PROFILE,TOTAL, SERUM (CLIA)			
T3, Total*	0.94	ng/mL	0.94 - 2.41
T4, Total*	6.90	µg/dL	6.40 - 13.30
TSH*	1.34	µIU/mL	0.70 - 6.40

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
2. Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
3. Unbound fraction (Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
4. Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals



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Test Name	Results	Units	Bio. Ref. Interval
LIPID SCREEN, SERUM			
Cholesterol, Total* (CHO-POD)	195.00	mg/dL	<170.00
Triglycerides* (GPO-POD)	75.00	mg/dL	<150.00
HDL Cholesterol* (Enzymatic Immunoinhibiti	44.00	mg/dL	40.00 - 60.00
LDL Cholesterol, Calculated	136.00	mg/dL	<110.00
VLDL Cholesterol,Calculated	15.00	mg/dL	<30.00
Non-HDL Cholesterol (Calculated)	151	mg/dL	<130

Interpretation

REMARKS	TOTAL CHOLESTEROL in mg/dL	TRIGLYCERIDE in mg/dL	LDL CHOLESTEROL in mg/dL
Optimal	<170	<150	<110
Borderline High	171-199	150-199	111-129
High	>=200	200-499	>=130
Very High	-	>=500	-

Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- ATP III recommends a complete lipoprotein profile as the initial test for evaluating cholesterol.
- Friedewald equation to calculate LDL cholesterol is most accurate when Triglyceride level is <400 mg/dL. Measurement of Direct LDL cholesterol is recommended when Triglyceride level is >400 mg/dL.



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Test Name	Results	Units	Bio. Ref. Interval
IMMUNOGLOBULIN IgE, SERUM** (FEIA)	109.00	kUA/L	<71.00

- Note:** 1. Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
2. No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments

Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.

Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Uses

- Evaluation of children with strong family history of allergies and early clinical signs of disease
- Evaluation of children and adults suspected of having allergic respiratory disease to establish the diagnosis and define the allergens
- To confirm clinical expression of sensitivity to foods in patients with Anaphylactic sensitivity or with Asthma, Angioedema or Cutaneous disease
- To evaluate sensitivity to insect venom allergens particularly as an aid in defining venom specificity in those cases in which skin tests are equivocal
- To confirm the presence of IgE antibodies to certain occupational allergens

ALLERGY, INDIVIDUAL MARKER, COCKROACH, SERUM** (FEIA)	0.07	kUA/L	<0.35
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Interpretation

Quantitative result in kUA/L	Level of Allergen specific antibody	Symptom relation
<0.10	Undetectable	Unlikely
0.10-0.50	very low	Uncommon
0.50-2.00	Low	Low



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Test Name	Results	Units	Bio. Ref. Interval
2.00-15.00	Moderate	Common	
15.00-50.00	High	High	
50.00-100.00	very High	very High	
>100.00	very High	very High	

Note

- Sensitized patients show elevated levels of specific allergens. Generally the higher the kUA/L value, the more exposed the patient is to the allergen.
- All results should be interpreted in relation to the individual case history.

Comments

Cockroaches are cosmopolitan and are probably the oldest insect in the world. They are found in homes, hotels, restaurants, food plants, warehouses, pantries, furniture cracks, closets, stores, refrigerators and dish washers. Cockroach allergens have been identified as a potent source of allergen in house dust. Allergic individuals can be exposed to cockroach allergens by inhalation from living quarters and ingestion of contaminated food stuff. Hypersensitivity to cockroach allergens has been implicated as a factor in urban asthma. The allergenic components are usually the cast skins leading to immediate skin reactions, bronchoconstriction and perennial asthma.

Potential Cross Reactivity

Seen with different species of the genus, house dust mites, tropomyosin found in shrimps and other crustacean species.

ALLERGY, INDIVIDUAL MARKER, HOUSE DUST, SERUM** (FEIA)	0.05	kUA/L	<0.35
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Interpretation

QUANTITATIVE RESULT IN kUA/L	LEVEL OF ALLERGEN SPECIFIC ANTIBODY	SYMPTOM RELATION
<0.10	Undetectable	Unlikely
0.10-0.50	very low	Uncommon
0.50-2.00	Low	Low
2.00-15.00	Moderate	Common



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Test Name	Results	Units	Bio. Ref. Interval
15.00-50.00	High	High	
50.00-100.00	very High	very High	
>100.00	very High	very High	

Note

1. Sensitized patients show elevated levels of specific allergens. Generally the higher the kUA/L value, the more exposed the patient is to the allergen.
2. All results should be interpreted in relation to the individual case history.

Comments

The ingredients of house dust that can cause allergic reactions include molds, pets and human dander, cockroach waste and mites. Ordinary house dust is the commonest cause of allergic manifestations like Asthma and Upper respiratory tract infections. The waste products of mites and cockroaches continue to cause allergic symptoms even after the mites and cockroaches that produced them have been killed. House dust allergy is the commonest environmental factor leading to Asthma in children.



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STOOL EXAMINATION, ROUTINE; STOOL, R/E* (Manual Method, Light microscopy)			
Colour*	Dark Brown		Brown
Form and Consistency*	Semi Formed		Semi Solid
Mucus*	Absent		Absent
Visible Blood*	Absent		Absent
Reaction*	Acidic		Alkaline
Pus Cells*	0 - 2	/hpf	0 - 5
RBC*	Nil	/hpf	None Seen
Macrophages*	None Seen		None Seen
Trophozoites*	None Seen		None Seen
Cysts*	None Seen		None Seen
Helminthic Ova*	None Seen		None Seen
Larva*	None Seen		None Seen
Other Observations*	None Seen		None Seen



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URINE EXAMINATION, ROUTINE; URINE, R/E (Dipstick, Microscopy)			
Physical			
Colour	Light Yellow		Pale yellow
Specific Gravity	1.025		1.001 - 1.030
pH	5		5.0 - 8.0
Chemical			
Proteins	Negative		Negative
Glucose	Negative		Negative
Ketones	Negative		Negative
Bilirubin	Negative		Negative
Urobilinogen	Negative		Negative
Leucocyte Esterase	Positive		Negative
Nitrite	Negative		Negative
Microscopy			
R.B.C.	0-1 RBC/HPF		0.0 - 2.0 RBC/hpf
Pus Cells	6 - 8 WBC/HPF		0-5 WBC / hpf
Epithelial Cells	3-5 Epi Cells/hpf		0.0 - 5.0 Epi cells/hpf
Casts	None seen		None seen/Lpf
Crystals	None seen		None seen
Others	None seen		None seen



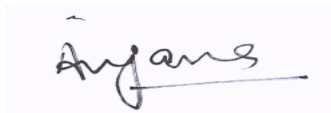
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Dr Ajay Gupta
MD, Pathology
HOD Hematology &
Immunohematology
NRL - Dr Lal PathLabs Ltd



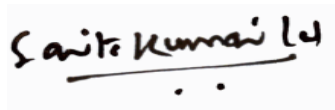
Dr Anjana Mittal
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Dr Jatin Munjal
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Consultant Pathologist
Dr Lal PathLabs Ltd



Dr Jasmine Kohli
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd



Dr Sarita Kumari Lal
MD, Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd



Dr Sunanda
MD, Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd

-----End of report -----

* Test conducted under NABL scope MC-2696,LPL-JAIPUR LAB at JAIPUR

** Test conducted under NABL scope MC-2113,LPL-NATIONAL REFERENCE LAB at NEW DELHI

IMPORTANT INSTRUCTIONS

- Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory.
- Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Sample repeats are accepted on request of Referring Physician within 7 days post reporting. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes.
- Contact customer care Tel No. +91-11-39885050 for all queries related to test results.
- (#) Sample drawn from outside source.

